



FREQUENTLY ASKED QUESTIONS — SURGERY AND PRE-OPERATIVE CARE

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பராமரிப்பு

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Dept: Surgical Centre, Levels 6 and 7, Block C | Valid Until: 31

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BEFORE SURGERY

How do I schedule a surgical procedure?

Surgery is scheduled through the treating consultant's office following an outpatient consultation and any required pre-operative investigations. Once the consultant confirms surgical clearance, the Surgical Admissions Desk on Level 6, Block C will contact the patient to assign a theatre date. Patients may also contact the Surgical Helpline directly at 0112-456-800.

What investigations are required before surgery?

Pre-operative investigations vary by procedure and patient history but typically include a full blood count, blood group and crossmatch, ECG, chest X-ray, and relevant biochemistry. The treating consultant will specify the exact panel required. All results must be submitted and cleared by the consultant at least 48 hours before the scheduled theatre date.

Should I stop any medications before surgery?

Certain medications require adjustment or temporary discontinuation before surgery. These commonly include anticoagulants such as warfarin and aspirin, antidiabetic agents including insulin, and some antihypertensive drugs. Patients are advised not to stop any medication without explicit instruction from the treating surgeon or the pre-anaesthesia assessment team (Ext. 2210). Herbal preparations and dietary supplements should also be disclosed at the pre-anaesthesia assessment.

Can I eat or drink before surgery?

Patients scheduled for general anaesthesia must remain nil by mouth from midnight on the night before surgery. This means no food, drink, chewing gum, or sweets. Clear water may be permitted up to two hours before the procedure in some cases; this will be confirmed by the anaesthesiology team at the pre-anaesthesia assessment. Failure to observe fasting instructions may result in postponement of the procedure for patient safety.

When do I meet the anaesthetist?

A pre-anaesthesia assessment is conducted by a member of the Anaesthesiology team at least 48 hours before the scheduled procedure. This appointment may be arranged through Ext. 2210. The assessment covers medical history, allergy status, current medications, airway evaluation, and fasting instructions. Patients with complex medical histories may require an earlier assessment.

What should I bring on the day of surgery?

Patients are requested to bring their National Identity Card or passport, pre-operative investigation results (originals and one copy), current medications in original packaging, insurance card and pre-authorisation letter if applicable, and comfortable loose clothing for post-operative recovery. Valuables, jewellery, and nail polish should be left at home. Dentures, hearing aids, and contact lenses must be removed before entering the theatre.

ON THE DAY AND AFTER

Who can accompany me on the day of surgery?

One responsible adult must accompany the patient on the day of surgery and remain on hospital premises until the patient is discharged from the recovery room. This person will be the primary contact for any updates from the surgical team. For day-case procedures, the accompanying adult must be available to transport the patient home, as patients may not drive or travel unaccompanied following anaesthesia.

What is day-case surgery?

Day-case surgery refers to procedures where the patient is admitted, operated upon, monitored in the recovery room, and discharged on the same calendar day without an overnight stay. Common day-case procedures at Serendib include cataract surgery, minor orthopaedic procedures, and selected laparoscopic operations. Eligibility for day-case surgery is determined by the surgeon and anaesthetist based on the procedure and the patient's medical status.

Will I experience pain after surgery?

Post-operative pain is expected and is actively managed by the clinical team. A post-operative pain management plan is prepared for each patient prior to surgery. Patients are encouraged to report pain levels honestly to the nursing staff, as adequate pain control supports faster recovery. The pain management team may adjust medication during the post-operative period based on the patient's response.

When can I return to normal activities after surgery?

Return to activity timelines vary significantly by procedure. As a general guide, minor procedures may allow return to light activity within one to two weeks, whereas major joint replacements or cardiac surgery may require six to twelve weeks of graduated rehabilitation. The treating surgeon will provide a personalised recovery plan and restrictions at discharge. Patients should not return to driving, operating machinery, or strenuous physical activity without the surgeon's explicit clearance.

How do I care for my wound after discharge?

Wound care instructions are provided in writing at discharge. In general, the wound should be kept clean and dry, dressings changed as directed, and signs of infection (redness, swelling, warmth, discharge, or fever) reported promptly to the Surgical Helpline at 0112-456-800. Suture or staple removal dates will be specified in the discharge summary.

What if I feel unwell after discharge from surgery?

Patients experiencing post-operative fever above 38.5°C, wound discharge, unusual or worsening pain, shortness of breath, swelling of the limbs, or any other concerning symptom after discharge are advised to contact the Surgical Helpline at 0112-456-800 before presenting to Accident and Emergency. The helpline is staffed 24 hours and will direct the patient appropriately.

Can I choose my surgeon?

Patients are free to request a specific surgeon at the time of consultation. The surgeon's availability and specialisation in the relevant procedure will be confirmed by the consultant's office. Where a specific surgeon is unavailable, the patient will be offered an alternative of equivalent seniority and subspecialty experience.

C. Wickramasinghe

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